TEST REPORT

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Phone: 503-466-2445 Fax: 503-466-1636

ZRT LABORATORY

 # 2025 02 21 035 S

Ordering Provider:
Ordrs

Reference Code
j9v7p0v

Samples Received
02/21/2025

Report Date
02/25/2025

Samples Collected
Saliva - 02/14/25 08:00
Saliva - 02/14/25 12:00
Saliva - 02/14/25 18:00
Saliva - 02/14/25 23:00

Patient Name: Jennifer Pei
Patient Phone Number: 917 743 1266

Gender Female	**Last Menses** Unspecified	**Height** Unspecified	**Waist** Unspecified
DOB 2/15/1994 (30 yrs)	**Menses Status** Pre-Menopausal	**Weight** Unspecified	

TEST NAME	RESULTS | 02/14/25	RANGE
Salivary Steroids		
Cortisol	9.2	3.7-9.5 ng/mL (morning)
Cortisol	4.5 H	1.2-3.0 ng/mL (noon)
Cortisol	6.6 H	0.6-1.9 ng/mL (evening)
Cortisol	1.7 H	0.4-1.0 ng/mL (night)

*<dl = Less than the detectable limit of the lab. **N/A** = Not applicable; 1 or more values used in this calculation is less than the detectable limit. **H** = High. **L** = Low.*

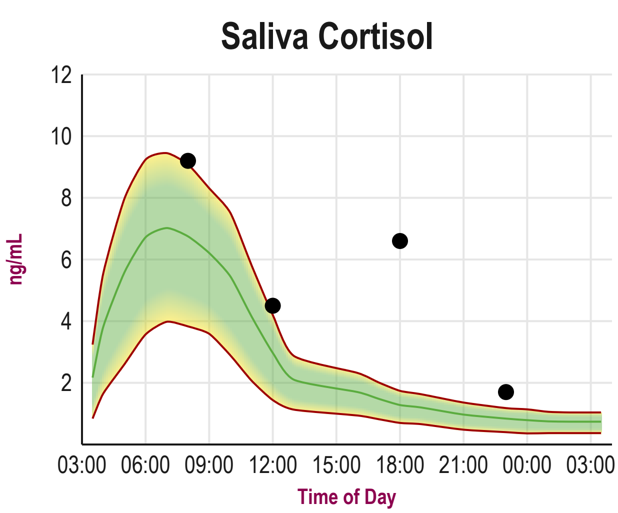
Therapies

None Indicated

Graphs

Disclaimer: Graphs below represent averages for healthy individuals not using hormones. Supplementation ranges may be higher. Please see supplementation ranges and lab comments if results are higher or lower than expected.

Average ▼▲ Off Graph



The above results and comments are for informational purposes only and are not to be construed as medical advice. Please consult your healthcare practitioner for diagnosis and treatment.

David T. Zava, Ph.D.
Laboratory Director

Alison McAllister, ND.
(Ordering Provider unless otherwise specified on page 1)

Jennifer Pei
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Lab Comments

CORTISOL (4x diurnal immunoassay) is not following a normal circadian rhythm and instead is flat with a lower level midday and high at night. Cortisol synthesis by the adrenal glands may be suppressed during midday due to medications that suppress adrenal cortisol synthesis or insufficient nutrients necessary for optimal cortisol synthesis (vitamins B5, A, C and adequate protein in the diet). The slightly higher night cortisol may be a rebound from cortisol suppression by medications or herbal supplements taken early during the day following the morning saliva collection or may result from some form of adrenal stressor(s) at night that stimulates adrenal gland synthesis of cortisol. This circadian pattern may also be typical of an individual on night shift work if the morning collection is very early. High night cortisol, and a blunted circadian rhythm (low morning cortisol) is the most problematic profile. High night cortisol is often associated with self-reported symptoms/conditions such as psychological stress (emotional) over unresolved situations, physical insults (pain, injury), exposure to toxic chemicals, hypoglycemia (low blood sugar), and pathogenic infections (bacterial, viral, fungal). While high cortisol production by the adrenal glands is a normal acute response to stressors and is essential for health, chronic stressors often lead to adrenal gland exhaustion and eventual low or flattened circadian cortisol levels. Chronic high cortisol, particularly high night cortisol, is often associated with symptoms/conditions of sleep disturbances, fatigue, depression, anxiety, and weight gain in the waist. When adrenal cortisol output remains high over months/years, this can lead to excessive breakdown of normal tissues (muscle wasting, thinning of skin, bone loss) and immune suppression. For additional information about strategies for supporting adrenal health and reducing stress(ors), the following books are worth reading: "Adrenal Fatigue", by James L. Wilson, N.D., D.C., Ph.D.; "The Cortisol Connection", by Shawn Talbott, Ph.D.; "The End of Stress As We Know It" by Bruce McEwen; "The Role of Stress and the HPA Axis in Chronic Disease Management" by Thomas Guilliams, PhD.